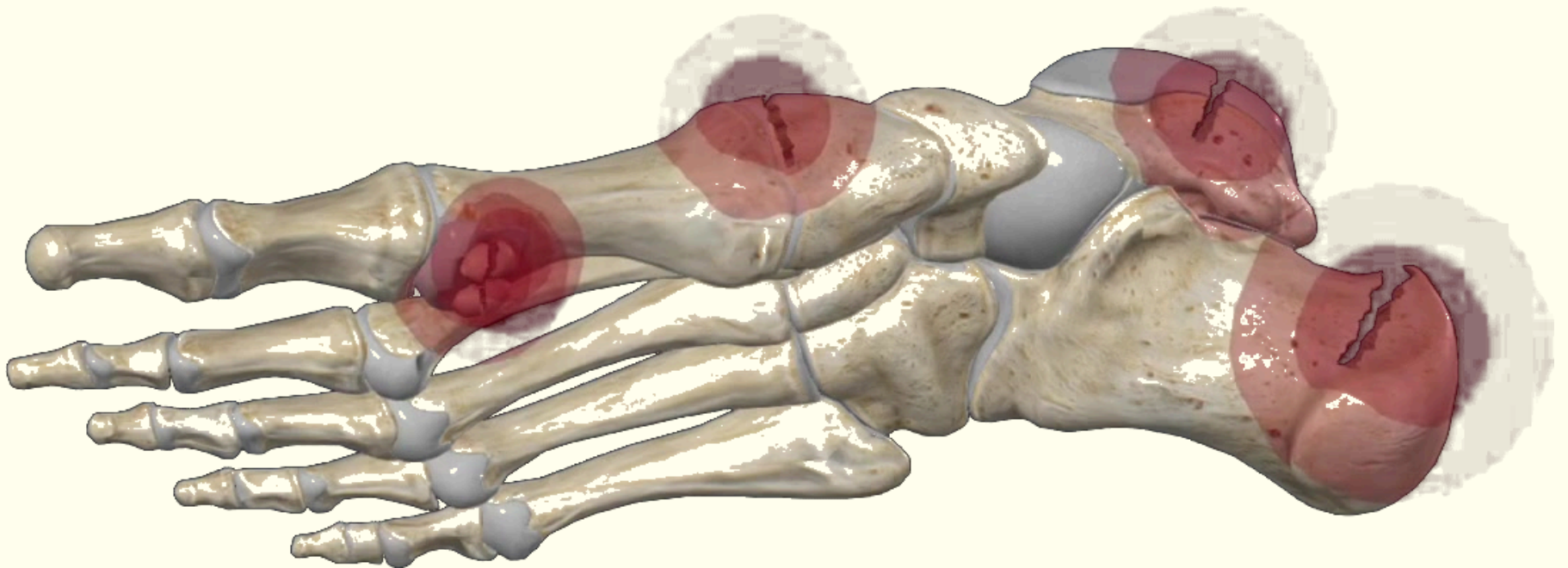


SAME SORE SPOT. VERY DIFFERENT RISK.

Base of 5th metatarsal bone stress, or peroneal tendinopathy. If symptoms were reliable, this mix up would not keep happening.



WHAT THEY **SHARE**

Pain on the outside of the foot

Worse with push off

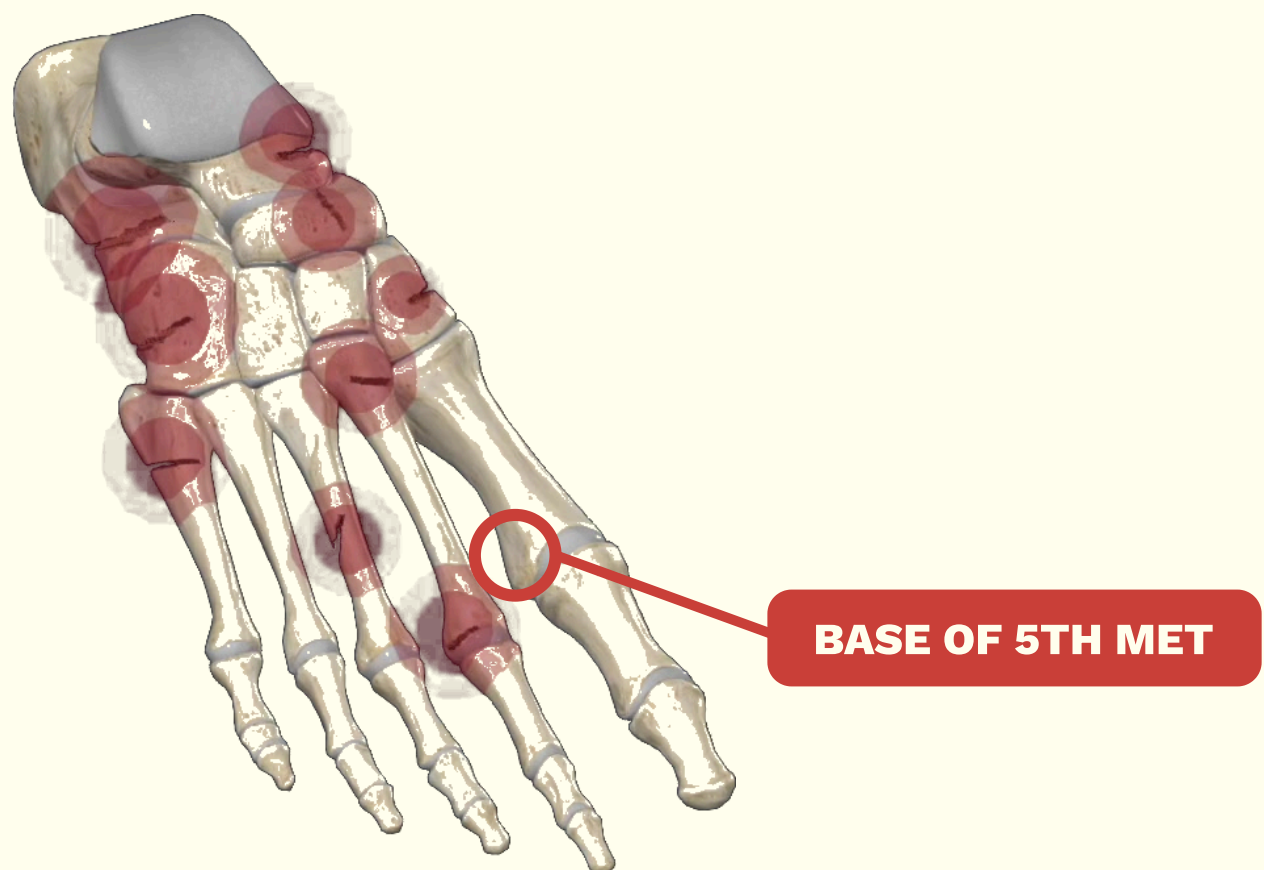
Often a gradual start, no single moment

Both settle a little with rest, then return

Symptoms alone will not separate these two.

BUT ONE OF THEM IS A HIGH RISK BONE

The base of the 5th metatarsal sits in a zone with a poor blood supply. That is why it carries higher rates of refracture and of not knitting together at all.



Treating it as a tendon problem and running through it is how these get worse.

WHAT ACTUALLY SEPARATES THEM

5TH MET BONE STRESS

- | Point tenderness right on the bony base
- | Sore with a single leg hop
- | Pain when the foot is rolled inward
- | Sometimes a palpable bony thickening

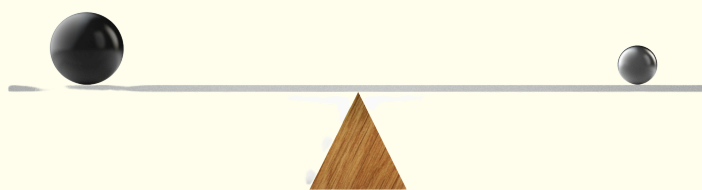
PERONEAL TENDINOPATHY

- | Tender along the line of the tendon
- | Sore with resisted eversion
- | Swelling behind the outer ankle bone
- | Worse with ankle instability tasks

AND THE PERSON MATTERS AS MUCH AS THE SPOT

The same lateral foot pain reads differently depending on who is carrying it.

A previous bone stress injury, a stretch of under fuelling, a sharp jump in training, or pain that never really behaves like a tendon. Any of those shift the odds toward the bone.



WHEN TWO THINGS FEEL THE SAME BUT CARRY **DIFFERENT RISK**

How it feels is a weak way to choose between them.
The site matters more, and so does the person
standing on it.



More on that soon.